

USER INFORMATION
READ CAREFULLY
HEMACOM AB
CONTINUOUS RENAL REPLACEMENT THERAPY SOLUTION
Sterile and Non-Pyrogenic

Description:

HEMACOM AB is a sterile clear and colourless solution. The solution is used in Continuous Renal Replacement Therapies (CRRT) for hemofiltration, hemodiafiltration and haemodialysis.

HEMACOM AB is packaged in a two compartment bag consist of compartment A and B. The compartment A contains electrolyte and compartment B contains buffer. Both compartments A and B to be mixed before use to obtain the final 5000mL reconstituted solution.

BEFORE RECONSTITUTION				FINAL ELECTROLYTE COMPOSITION (AFTER RECONSTITUTION A + B)		
Compartment A (250mL):		Compartment B (4750mL):				
Ingredients	g/L	Ingredients	g/L	Electrolytes	mmol/L	mEq/L
Calcium Chloride 2H ₂ O BP	5.15	Sodium Chloride BP	6.46	Sodium Ion (Na ⁺)	140.00	140.00
Magnesium Chloride 6H ₂ O BP	2.03	Sodium Bicarbonate BP	3.09	Calcium Ion (Ca ⁺⁺)	1.75	3.50
Lactic Acid	5.40			Magnesium Ion (Mg ⁺⁺)	0.50	1.00
				Chloride Ion (Cl ⁻)	109.50	109.50
				Lactate	3.00	3.00
				HCO ₃ ⁻	32.00	32.00
				Calc. Osmolarity	287 mOsm/L	

Pharmacodynamics:

HEMACOM AB is pharmacologically inactive. The sodium, calcium, magnesium and chloride ions are present at concentrations similar to physiological levels in plasma. The solution is used to replace water and electrolytes removed during haemofiltration or to serve as a suitable exchange medium for use during haemodiafiltration or continuous haemodialysis. Bicarbonate is used as an alkalinising buffer.

Pharmacokinetics:

Not relevant. The active ingredients are pharmacologically inactive and are present at concentrations similar to physiological plasma levels.

Indication:

As substitution solution in continuous haemofiltration and haemodiafiltration and as dialysis solution in continuous haemodialysis for acute renal failure.

Dosage:

The volume of substitution solution to be administered will depend on the intensity of the treatment performed and on the amount of solution which has to be replaced in order to achieve the target fluid balance. The dose volume is therefore at the discretion of the responsible physician.

Commonly used flow rates for the substitution solution in haemofiltration and haemodiafiltration are:

Adult: 500 - 1500 mL/hour
Children: 15 - 20 mL/kg/hour

Commonly used flow rates for the dialysis solution (dialysate) in continuous haemodialysis are:

Adult: 500 - 2000 mL/hour
Children: 15 - 20 mL/kg/hour

HEMACOM AB, when used as a substitution solution is administered into the circuit before (predilution) or after the haemofilter (post-dilution).

Contraindications:

Hypokalaemia Metabolic alkalosis.

Warnings and Precautions :

HEMACOM AB should be mixed before use by mixing solution from Compartment A and B. Use within 24 hours after reconstitution. The haemodynamic status, fluid, electrolyte and acid-base balance should be closely monitored during the CRRT procedure. Before use check for minute leaks by squeezing bag firmly. Discard if leakage is detected. If any solid particle, growth or turbidity appears during storage, the product should not be used. For single use only. Discard unused portion. Not for direct infusion. The administration of CRRT has to be strictly carried out under aseptic condition.

Interactions with Other Medicaments:

Some medicament may be incompatible with HEMACOM AB. The compatibility should be checked before use. Generally, other medicament should be administered through different line.

The blood concentration of filterable/dialysable drugs may be reduced during treatment. Corresponding corrective therapy should be instituted if necessary. Interactions with other medications due to electrolyte and/or acid-base imbalances can be avoided by correct dosage of the solution for haemofiltration and haemodialysis and precise monitoring. However, the following interactions are conceivable:

- The risk of digitalis-induced cardiac arrhythmia is increased during hypokalaemia;
- Vitamin D and medicinal products containing calcium, e.g. calcium carbonate as phosphate binder, can increase the risk of hypercalcaemia;
- Additional sodium bicarbonate substitution may increase the risk of metabolic alkalosis.

Pregnancy and Lactation:

There are no adequate data from the use of HEMACOM AB in pregnant or lactating women. The prescriber should consider the benefit/risk relationship before administering HEMACOM AB to pregnant or breast-feeding women.

Side Effects/Adverse Reactions:

Adverse reactions can result from the solution or the CRRT procedure. Improper use can lead to fluid imbalance and disturbances in electrolyte, acid-base and glucose balance.

Some undesirable effects related to the dialysis treatment can occur, such as nausea, vomiting, muscle cramps and hypotension. Electrolytes disturbances may occur. Special attention must be taken for patients with hypokalaemia as this solution is potassium-free.

Overdose and Treatment:

Overdose should not occur if the procedure is carried out correctly and the fluid balance, electrolyte and acid-base balance of the patient are carefully monitored. However, overdose will result in fluid overload in patients with renal failure. Continued application of haemofiltration will remove excess fluid and electrolytes. In case of hyperhydration, the ultrafiltration must be increased and the rate of administration of the solution for haemofiltration reduced. In the case of a severe dehydration it is necessary to cease ultrafiltration and to increase the inflow of solution for haemofiltration appropriately. Overdose could lead to severe consequences, such as congestive heart failure, electrolyte or acid-base disturbances.

Storage Condition Before and After Reconstitution:

Do not store above 30°C

Shelf Life:

1 year from manufacturing date in proper storage condition. The solution should be used within 24 hours after reconstitution. Do not use after expiry.

Presentation:

5000mL x 1 Double Compartment PVC Collapsible Bag per Carton
5000mL x 2 Double Compartment PVC Collapsible Bags per Carton
5000mL x 1 Double Compartment PP Collapsible Bag per Carton
5000mL x 2 Double Compartment PP Collapsible Bags per Carton

Manufacturer/Product Registration Holder :

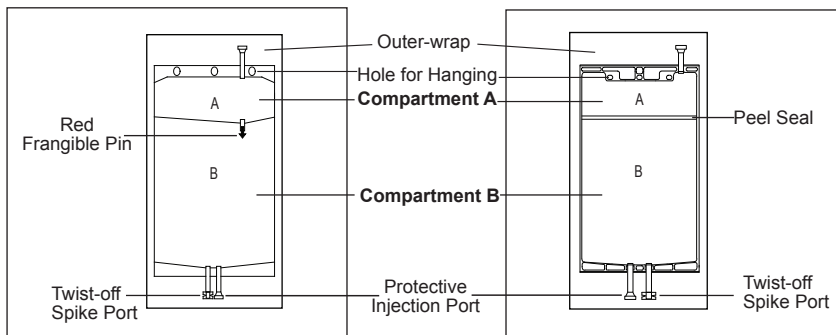
 **AIN MEDICARE SDN. BHD.**
Jalan 6/44, Kaw. Perindustrian Pengkalan Chepa 2, 16100 Kota Bharu, Kelantan, MALAYSIA.

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HANDLING INSTRUCTION

AIN MEDICARE CRRT FLUID IN DOUBLE COMPARTMENT COLLAPSIBLE BAG

PARTS OF THE DOUBLE COMPARTMENT COLLAPSIBLE BAG



Design 1: PVC Collapsible Bag

Design 2: PP Collapsible Bag

Handling for PVC Collapsible Bag

Handling for PP Collapsible Bag

1	Remove the outerwrap from the bag immediately before use and discard any other packaging materials. Break the red frangible pin between the two compartments to allow the solution to mix. The frangible pin will remain in the bag.	1	Remove the outerwrap from the bag immediately before use. Open the seal by holding the compartment A with both hands and squeeze it until an opening is created in the peel seal between the two compartments.
2	Make sure all the fluid from compartment A is completely transferred into compartment B. After compartment A is empty, rinse the compartment A TWICE by pressing the mixed solution back into compartment A and then back into compartment B.	2	Push with both hands on the large compartment until the peel seal between the two compartments is entirely open.
3	When compartment A is empty; shake the bag so that the contents mix completely. The solution is now ready to use, and can hung on the equipment.	3	Secure complete mixing of the solution by shaking the bag gently. the solution is now ready for use, and can be hung on the equipment.

4	Additives may be injected through injection port. • Remove the aluminium cap. • Disinfect the injection port. • Hold the injection port and inject through the rubber piece. • Mix thoroughly with the solution.
5	Twist-off the tab to expose the spike-port.
6	Insert spike of the administration set into the spike port aseptically.